

1. Administrative & Study Identification

Brief Study Title: A Global Study to Assess the Effects of Durvalumab + Domvanalimab Following Concurrent Chemoradiation in Participants With Stage III Unresectable NSCLC **Official Full Study Title:** A Phase III, Randomised, Double-blind, Placebo-controlled, Multicentre, International Study of Durvalumab plus Domvanalimab (AB154) in Participants with Locally Advanced (Stage III), Unresectable Non-small Cell Lung Cancer Whose Disease has not Progressed Following Definitive Platinum-based Concurrent Chemoradiation Therapy **Short Title/Acronym:** PACIFIC-8 **Protocol Number:** D9075C00001 **NCT Number:** NCT05211895 **Company / Sponsor Name:** AstraZeneca **Investigational Product:** Durvalumab (MEDI4736, Trade Name: Imfinzi) + Domvanalimab (AB154) **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy and safety of durvalumab (MEDI4736) and domvanalimab (AB154) compared with durvalumab plus placebo in adults with locally advanced (Stage III), unresectable NSCLC whose disease has not progressed following definitive platinum-based cCRT. **Secondary Objectives:** To evaluate PFS, Overall Survival (OS), Objective Response Rate (ORR), Duration of Response (DoR), safety and tolerability profiles, and patient-reported outcomes.

2.2 Background & Rationale Consolidation durvalumab following no progression on concurrent chemoradiotherapy (cCRT) is the current standard of care for unresectable stage III non-small-cell lung cancer (NSCLC). To improve outcomes further in this population, novel immunotherapy combinations that build on the backbone of PD-L1 inhibition are being explored. Domvanalimab (AB154) is a novel anti-TIGIT monoclonal antibody. Combining durvalumab with domvanalimab may further enhance the ability of the immune system to recognize and kill cancer cells by blocking a secondary inhibitory pathway.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** 1:1

3.2 Planned Enrollment & Duration Trial Status: RECRUITING (Status Description: Currently recruiting participants) **Targeted Enrollment:** 860 participants **Number of Sites:** International, ~22

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participant must be ≥ 18 years at the time of screening. 2. Histologically- or cytologically- documented NSCLC and have been treated with concurrent CRT for locally advanced, unresectable (Stage III) disease. 3. Provision of a tumour tissue sample obtained prior to CRT. 4. Documented tumour PD-L1 status $\geq 1\%$ by central lab. 5. Documented EGFR and ALK wild-type status (local or central). 6. Patients must not have progressed following definitive, platinum-based, concurrent chemoradiotherapy. 7. Participants must have received at least 2 cycles of platinum-based chemotherapy concurrent with radiation therapy. 8. Participants must have received a total dose of radiation of 60 Gy $\pm 10\%$ (54 Gy to 66 Gy) as part of the chemoradiation therapy, to be randomised. Radiation therapy should be administered by intensity modulated RT (preferred) or 3D-conforming technique. 9. WHO performance status of 0 or 1 at randomization. 10. Adequate organ and marrow function.

4.2 Exclusion Criteria 1. History of another primary malignancy, except for: * Malignancies treated with curative intent and adequate follow-up with no known active disease and have not required active treatment within the past 3 years before the first dose of study intervention and of low potential risk of recurrence. * Adequately resected non melanoma skin cancer or lentigo maligna without evidence of disease. * Adequately treated carcinoma in situ, including Ta tumors without evidence of disease. 2. Mixed small cell and non-small cell lung cancer histology. 3. Participants who receive sequential (not inclusive of induction) chemoradiation therapy for locally advanced (Stage III) unresectable NSCLC. 4. Participants with locally advanced (Stage III) unresectable NSCLC who have progressed during platinum-based cCRT. 5. Any unresolved toxicity CTCAE $>$ Grade 2 from the prior chemoradiation therapy (excluding alopecia). 6. Participants with $\geq$ grade 2 pneumonitis from prior chemoradiation therapy. 7. History of idiopathic pulmonary fibrosis, drug-induced pneumonitis, or idiopathic pneumonitis - regardless of time of onset prior to randomisation. Evidence of active non-CRT induced pneumonitis ($\geq$ Grade 2), active pneumonia, active ILD, active or recently treated pleural effusion, or current pulmonary fibrosis. 8. Active or prior documented autoimmune or inflammatory disorders (with exceptions). 9. Active EBV infection, or known or suspected chronic active EBV infection at screening. 10. Current or prior use of immunosuppressive medication within 14 days before the first dose of durvalumab.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Investigational Product 1: durvalumab (Drug Preferred Name: durvalumab; Trade Names: Imfinzi; ATC Code: L01FF03; EMA URL: https://www.ema.europa.eu/en/documents/product-information/imfinzi-epar-product-information_en.pdf) **Dosage/Frequency:** Durvalumab 1500 mg combined with either domvanalimab 20 mg/kg or placebo, administered every 4 weeks. **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Up to 12 months.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care concomitant therapies for symptom management. **Prohibited:** Systemic immunosuppressive medications (e.g., corticosteroids > 10 mg/day prednisone equivalent) or concurrent investigational anti-cancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor tissue testing (PD-L1, EGFR/ALK), Baseline Imaging
Baseline	Day 0	Randomization, First Dose of Durvalumab + Domvanalimab/Placebo
Treatment	Q4W	Safety Labs, PK/ADA collection
Interim	Q8W to Q12W	Efficacy Assessment via Imaging (CT/MRI) per RECIST v1.1
End of Study	Month 12	End of Treatment Visit, Survival Follow-Up phase initiation

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Progression Free Survival (PFS): Defined as time from randomisation until progression per RECIST 1.1 as assessed by Blinded Independent Central Review (BICR), or death due to any cause in participants with PD-L1 TC $\geq 50\%$.

7.2 Secondary Endpoints 1. Progression Free Survival (PFS) 2. Overall Survival (OS) 3. Objective Response Rate (ORR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine collection via HCP assessment, physical exams, and lab evaluations per CTCAE v5.0 guidelines. Specific monitoring for immune-mediated adverse events (imAEs) such as pneumonitis. **Serious Adverse Events (SAEs):** Required to be reported to the sponsor within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) utilized to oversee safety signals throughout the global trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary and secondary efficacy endpoints; Safety Set for adverse events analyses. **Sample Size Justification:** 860 patients randomized 1:1, powered to detect a statistically significant improvement in PFS and OS in the targeted PD-L1 subgroups.

Handling of Missing Data: Standard survival analysis methodologies applying censoring rules for time-to-event variables (e.g., PFS, OS) at the last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure protocol adherence and data integrity. **Audit Trail:** Robust Electronic Data Capture (EDC) system with full audit trails, strict eCRF locking mechanisms, and automated data clarification forms.

11. Study Identifiers & Governance

Primary ID: D9075C00001 **Registry Number:** NCT05211895 **Posting ID:** 456367131237391944 **Sponsor/Lead Organization:** AstraZeneca
Criteria Type / Text Requirement: PHASE_REQUIREMENT (Trial must be in PHASE3 / Phase ID: PHASE3) **Source Level:** 5

12. Clinical Framework (Terminology & Identifiers)

Disease Areas: Non-Small Cell Lung Cancer **Condition Name:** Metastatic Non Small Cell Lung Cancer **Condition Preferred Name:** Metastatic non-small cell lung cancer **Preferred UMLS Name:** Non-Small Cell Lung Carcinoma **Semantic Type:** Neoplastic Process **MeSH Code:** D016938 **SNOMED ID:** 457721000124104 **Definition:** Stage IV includes: (Any T, Any N, M1a); (Any T, Any N, M1b). M1a: Lung cancer with separate tumor nodule(s) in a contralateral lobe; tumor with pleural nodules or malignant pleural (or pericardial) effusion. M1b: Distant metastasis. (AJCC 7th ed.)

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathway post-cCRT
Education Protocol (HCP): Investigator protocol training, immune-mediated AE management guidelines **Education Protocol (Patient):** Informed consent, immunotherapy side effect recognition **Quality Audit Mechanism:** Centralized imaging review (BICR) and biomarker testing by central laboratories

14. Observation & Follow-Up Schedule

Total Duration: Up to 12 months of active treatment + Long-term survival follow-up **Onsite Visit Cadence:** Every 4 weeks (Q4W) for IV infusions **Remote Monitoring Frequency:** Routine survival calls post-treatment phase **Checklist Review Interval:** Restaging scans every 8 to 12 weeks per standard oncological protocols

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					